The Ohio State University Wexner Medical Center

Department of Pathology · Surgical Pathology

SURGICAL PATHOLOGY REPORT

PRELIMINARY · CONSULTATION PENDING

Patient	SMITH, JANE	**Accession**	PV26-001
DOB / Age / Sex	03/14/2012 · 14 y · F	**MRN**	DEMO-000001
Collected	10/06/2026	**Received**	10/06/2026
Specimen	Left forearm, soft tissue mass, excision	**Preliminary report**	10/09/2026
Ordering service	Orthopaedic Oncology (demo)	**Copy to**	Archana Shenoy, MD (consultant)

PRELIMINARY DIAGNOSIS

LEFT FOREARM, SOFT TISSUE MASS, EXCISION:
– HIGH-GRADE ROUND CELL SARCOMA, SUSPICIOUS FOR CIC-REARRANGED SARCOMA (SEE COMMENT).
– TUMOR SIZE: 3.1 CM (GROSS). TUMOR IS 0.3 CM FROM THE DEEP INKED MARGIN.
– FINAL CLASSIFICATION PENDING EXPERT CONSULTATION AND MOLECULAR STUDIES.

COMMENT

The morphologic and immunohistochemical findings are those of a high-grade round cell sarcoma. CD99 shows patchy staining rather than the strong, diffuse membranous pattern typical of Ewing sarcoma, and ETV4 and WT1 show nuclear expression. This immunophenotype raises concern for CIC-rearranged sarcoma. DUX4 immunohistochemistry and molecular testing are not available at this institution, and expert consultation has been requested (see below). The tumor area has been annotated on the digital slide and block A3 marked for macrodissection. An addendum or final report will follow.

CONSULTATION REQUESTED

Consultant	Archana Shenoy, MD · Department of Pathology, Cincinnati Children's Hospital Medical Center
Question	Confirmation of suspected CIC-rearranged sarcoma; DUX4 immunohistochemistry and molecular testing requested.
Sent	10/09/2026, digitally via PathPresenter. No glass shipped.
Materials	Whole-slide images of H&E slides A1–A4 and CD99, ETV4 and WT1 (block A3); this preliminary report; clinical history; prior biopsy PV26-000 (slides and report).
Molecular	Not available at this institution. Block A3 marked for macrodissection (tumor area annotated on the digital slide; estimated tumor content 70%; necrosis under 10% in the selected area).

CLINICAL HISTORY

14-year-old girl with a slowly enlarging, painless nodule on the left forearm over six months. A prior core biopsy was inconclusive (PV26-000). Imaging: not provided with the requisition.

PRIOR MATERIAL

PV26-000, core needle biopsy, left forearm (09/2026): atypical round cell proliferation; insufficient for definitive classification. Slides reviewed in conjunction with the current specimen.

The Ohio State University Wexner Medical Center
Department of Pathology · Surgical Pathology

SURGICAL PATHOLOGY REPORT
PRELIMINARY · CONSULTATION PENDING

GROSS DESCRIPTION

Received in formalin, labeled with the patient's name and “left forearm mass,” is an unoriented 4.2 × 3.6 × 2.9 cm portion of soft tissue. The surface is inked black. Sectioning reveals a well-circumscribed, tan-white, fleshy mass measuring 3.1 × 2.7 × 2.4 cm, with focal hemorrhage and necrosis (approximately 15%). The mass is 0.3 cm from the deep inked margin and 0.8 cm from the closest peripheral margin. Representative sections are submitted in cassettes A1–A8: A1–A5, tumor including the deep margin; A6–A8, peripheral margins.

MICROSCOPIC DESCRIPTION

Sections show a lobulated, highly cellular neoplasm composed of sheets of small to medium-sized round cells with scant to moderate pale-to-eosinophilic cytoplasm, vesicular chromatin and conspicuous nucleoli. Mild to moderate nuclear pleomorphism is present, more than is usual for Ewing sarcoma. Areas of myxoid stroma separate the lobules. There is geographic necrosis and brisk mitotic activity. No rosettes, rhabdoid features or desmoplastic stroma are identified.

IMMUNOHISTOCHEMISTRY

Antibody	Result	Pattern / comment
CD99	Positive	Patchy; predominantly cytoplasmic with focal membranous staining; not diffuse
ETV4	Positive	Nuclear, in the majority of tumor cells
WT1	Positive	Nuclear
DUX4	Not performed	Not available at this institution; requested with consultation

Performed on block A3 with appropriate positive and negative controls. Immunohistochemical tests were developed and their performance characteristics determined by the performing laboratory.

Swati Satturwar, MD
Pathologist, The Ohio State University Wexner Medical Center
Electronically signed (preliminary): 10/09/2026 16:42 EDT

This is a preliminary report. A final report will be issued after expert consultation and molecular studies. Preliminary findings were communicated to the ordering service on 10/09/2026.